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**Accelerating digital transformation of health systems:  
Establishment of the South-East Asia digital health and  
artificial intelligence collaborative network (SEA-DAC)**

Digital transformation of health systems is a critical accelerator of universal health coverage (UHC) and the health-related Sustainable Development Goals (SDGs). The Global Initiative on Digital Health (GIDH), launched by the World Health Organization in February 2024 as a WHO-managed ‘network of networks’, provides the global framework to align resources with country-led digital transformation, reduce duplication and fragmentation, and strengthen knowledge exchange.

Member States in the South-East Asia Region have made considerable progress in advancing national digital health strategies, telemedicine, health information systems, digital public infrastructure (DPI) and early applications of artificial intelligence (AI). However, regional collaboration remains fragmented and largely ad hoc. Formal requests from Member States for structured peer learning and technology cooperation, together with standing offers of financial and technical support under GIDH, have been slowed by case-by-case bilateral coordination, underscoring the need for a dedicated, Member State-led regional mechanism.

This Working Paper proposes the establishment of the South-East Asia Digital Health and Artificial Intelligence Collaborative Network (SEA-DAC) as the dedicated regional implementation arm of GIDH. SEA-DAC would operate as a voluntary, consensus-based network of national agencies responsible for digital health and AI for health, governed by an Assembly of Members, a Steering Group, technical Working Groups and national focal points, supported by a WHO-SEARO Secretariat. Its design adopts validated practices from established global and regional mechanisms and is explicitly framed to complement, and not duplicate, existing partnerships, with technical delivery operationalized through WHO project collaboration agreements.

This Working Paper was presented to the High-Level Preparatory (HLP) Meeting for its review and recommendations. The HLP Meeting reviewed the paper, deliberated on the agenda item, and made the following recommendations for consideration by the Seventy-ninth Session of the Regional Committee:

**Actions by Member States**

- (1) Support the establishment of the South-East Asia Digital Health and Artificial Intelligence Collaborative Network (SEA-DAC) by the Regional Committee and, subject to endorsement, designate a national agency for digital health and nominate a senior representative and national focal points for its governance and technical work.
- (2) Identify and share national priorities, technical expertise, digital public goods and adaptable solutions that can contribute to peer learning, technology cooperation and early SEA-DAC deliverables.
- (3) Contribute actively to the governance, workplan and sustainable financing of SEA-DAC, while upholding equity, national sovereignty, responsible AI and complementarity with existing mechanisms.

**Actions by WHO**

- (1) Subject to the Regional Committee's endorsement and subsequent action as per the recommendations for Member States, convene the inaugural Assembly of Members of the Network.
- (2) Present the draft terms of reference and governance documents for consideration and adoption, as appropriate, by the inaugural Assembly of Members, and facilitate the establishment of the initial working groups and a results-oriented workplan.
- (3) Propose sustainable and transparent financing, due diligence, conflict-of-interest stipulations, monitoring and reporting arrangements, and priority early outputs for the Network.

The Working Paper, together with the recommendations of the HLP Meeting, is placed before the Seventy-ninth Session of the WHO Regional Committee for South-East Asia for its consideration and decision.

## Introduction

1. Digital transformation of health systems has emerged as one of the most powerful accelerators of universal health coverage and the health-related Sustainable Development Goals. Digital health, data and artificial intelligence are reshaping how health services are planned, delivered, monitored and evaluated — from telemedicine and electronic health records to integrated disease surveillance, digital public infrastructure (DPI) and AI-assisted clinical and public health decision-making. Realizing these benefits equitably, safely and sustainably requires deliberate governance, interoperable standards and sustained investment in institutional and workforce capacity.

2. The Global Strategy on Digital Health 2020–2027 and its successor Strategy 2028–2035 set the global policy direction, and the Global Initiative on Digital Health (GIDH), launched by WHO in February 2024, provides the delivery framework. GIDH is a WHO-managed “network of networks” organized around four pillars — the Country Needs Tracker, the Country Resource Portal, the Transformation Toolbox, and Convening and Knowledge Exchange — designed to align resources with country-led transformation, reduce duplication and fragmentation, and strengthen knowledge exchange.

3. In the South-East Asia Region, accelerating the digital transformation of health systems is a recognized regional priority, consistent with the regional strategic directions for 2026–2028 that identify digital health and AI as a key accelerator of health and well-being. Member States have repeatedly signalled demand for a more structured, regionally coordinated mechanism to translate global GIDH objectives into concrete, country-led action.

4. This Working Paper reviews progress and persisting challenges in regional digital health collaboration and proposes the establishment of the South-East Asia Digital Health and Artificial Intelligence Collaborative Network (SEA-DAC) — a Member State-led regional platform conceived as the dedicated regional implementation arm of GIDH. It summarizes the demand for such a network, its proposed design, governance and terms of reference, its complementarity with existing mechanisms, and a phased implementation roadmap, and proposes actions for consideration by Member States and WHO.

## Current situation, response and challenges

### Progress in the digital transformation of health systems

5. Member States in the Region have made considerable progress in advancing digital health. Most have adopted national digital health strategies, blueprints or roadmaps; expanded telemedicine services, particularly following the COVID-19 pandemic; strengthened national health information and disease surveillance platforms; and initiated national digital public infrastructure for health, including digital identity, health information exchange and interoperability programmes. Several Member States are piloting AI applications in clinical and public health settings and developing governance approaches for the safe, ethical and responsible use of AI in health.

6. Regional action has accompanied these national efforts. WHO, through its Regional Office for South-East Asia, has supported digital health maturity assessments, technical assistance, capacity-building activities and regional consultations, and has facilitated Member State engagement with GIDH. Member States have participated in regional and global digital health platforms, exchanged experiences through summits, workshops and study visits, and contributed to the development of regional norms and guidance.

## Member State demand for structured regional collaboration

7. Demand for a dedicated regional mechanism is explicit and growing. Member States have submitted formal requests through their ministries of health for peer learning, exposure visits and technology cooperation under GIDH, and several others have signalled willingness to participate in GIDH-aligned regional collaboration. Standing offers of support have also been made under GIDH, including a dedicated financial commitment to support digital public infrastructure and technology transfer across the Region, and offers of mature, customizable national digital health systems for replication and adaptation. Despite this demonstrated demand and supply, progress through ad hoc bilateral arrangements has been slowed by coordination, approval, logistical and resource-alignment challenges.

8. A Regional Open Digital Health Summit, convened in November 2025 in New Delhi jointly by the Government of India (Ministry of Health and Family Welfare and Ministry of Electronics and Information Technology), WHO-SEARO and UNICEF, brought together representatives of Member States of the Region to advance open, interoperable, standards-based digital health ecosystems.

9. During the Summit's session on regional collaboration, Member States reflected on a draft framework and proposed Terms of Reference for SEA-DAC, providing the consultative basis for the proposal presented in this Working Paper. The proposal was subsequently reviewed by the High-Level Preparatory Meeting for the Seventy-ninth Session of the Regional Committee, at which Member States expressed broad support for the establishment of SEA-DAC and emphasized voluntary participation, national ownership and sovereignty, standards and interoperability, ethical and responsible use of artificial intelligence and data governance, equity, capacity-building, sustainable financing and complementarity with existing global and regional mechanisms.

## Strategic and operational challenges

10. **Inefficient knowledge exchange and collaboration gaps.** Digital health — and particularly the fast-evolving domains of AI, machine learning and DPI — requires an agile, efficient and sustained learning platform. Member States are willing to share open-source and standards-based applications, yet the capacity for peer-to-peer knowledge transfer and the structured sharing of digital public goods remains underdeveloped. Country teams often plan in isolation, without a structured mechanism to share plans, identify overlaps or explore opportunities for collaboration.

11. **Diversity in digital health maturity.** Member States share many commonalities, but significant inter-country and intra-country variation exists in digital health maturity, resource availability and health-system priorities. This heterogeneity calls for a collaborative framework flexible enough to promote context-specific adaptation of international standards — such as a regional community profile of interoperability standards — and policy frameworks, rather than enforcing rigid uniformity.

12. **Limitations of ad hoc engagement models and bilateral bottlenecks.** Regional engagement currently relies heavily on periodic summits, conferences and workshops. While valuable for high-level dialogue, such events are insufficient for the deep, sustained technical convergence and capacity development required for long-term systems stability, and are vulnerable to funding constraints that limit continuity. Even where political will and financing exist, the transfer and adaptation of digital public infrastructure between countries faces regulatory, logistical and coordination hurdles. A mandated regional mechanism, supported by appropriate legal and operational instruments, is needed to convert standing offers of support into timely, equitable, country-led implementation.

### Complementarity with existing global and regional mechanisms

13. SEA-DAC is designed to complement, not duplicate, existing partnerships, and its design deliberately adopts their validated practices. From GIDH, it adopts the four-pillar architecture and its positioning as a WHO-supported regional implementation arm. From the Joint Learning Network (JLN) for universal health coverage, it adopts the country-led “joint learning” method and the co-development of practical knowledge products.

14. From the Global Digital Health Partnership (GDHP), it adopts co-chaired workstreams oriented towards concrete, reusable deliverables. From the South-East Asia Regulatory Network (SEARN), it adopts a proven regional governance architecture — Assembly of Members, Steering Group, Working Groups and a WHO-SEARO-supported Secretariat — together with consensus decision-making, focal-point system and a confidentiality and links-of-interest regime.

15. From the South-East Asia Regional Forum for PHC-oriented Health Systems, it adopts country-led knowledge-management, wider partnership and learning-visit practices. In relation to the Asia eHealth Information Network (AeHIN) and similar communities of practice, SEA-DAC is intended to leverage and reinforce — not replace — their strengths, adding a Region-specific, Member State-mandated decision-making layer with formal accountability to national health agencies; reciprocal participation and shared knowledge products are a design principle of the network.

### The proposed network: SEA-DAC

16. **Rationale.** A specialized regional digital health and AI network is justified by the need for focused, sustained, Member State-led action tailored to the Region: it develops solutions for the epidemiological, climatic and socioeconomic conditions common to the Region; it closes the gap between high-level global policy guidance and the detailed technical implementation required at national and subnational levels; it adopts a proven and validated governance model, enabling rapid establishment of a credible and sustainable operating structure; it places national health agencies at the centre of decision-making, giving the network legitimacy and continuity; and it provides a delivery route through which standing offers of support can be matched to country needs.

17. **Governance.** SEA-DAC would operate through a voluntary, consensus-based governance model comprising: an Assembly of Members (the highest decision-making body, comprising heads of designated national agencies responsible for digital health and AI for health , meeting at least annually); a Steering Group of senior representatives providing operational oversight between Assembly meetings; technical Working Groups executing the work plan under co-chairs drawn from different Member States; a Secretariat supported by WHO-SEARO providing coordination, documentation and communication support; and national focal points nominated by each Member. Decisions would be taken by consensus wherever possible. Guiding principles include country leadership, complementarity and non-duplication, equity and inclusion, respect for national sovereignty and legal frameworks, openness and preference for open standards and digital public goods, and transparency and sustainability.

18. **Initial Working Groups.** Four initial Working Groups are proposed for establishment by the first Assembly: (i) interoperability and standards, including the development and maintenance of a regional standards profile; (ii) digital public infrastructure, supporting the adaptation, reuse and joint deployment of health DPI and digital public goods and curating a regional Transformation Toolbox; (iii) AI, data governance and ethics, advancing trustworthy AI, privacy, security and the responsible deployment of AI in health; and (iv) capacity building, measurement and learning, coordinating fellowships, exchanges and learning visits and documenting best practices.

19. **Delivery through project collaboration agreements.** SEA-DAC is conceived as the governance and coordination layer of regional collaboration. Technical delivery to Member States — such as DPI and digital public goods transfer, technical assistance, joint deployments and capacity building — would be operationalized through project collaboration agreements and other appropriate instruments established at WHO-SEARO. Working Groups would capture country needs and match them to available resources and offers of support; WHO-SEARO would formalize the corresponding partnerships; and delivery would proceed under these agreements, with SEA-DAC providing oversight, alignment to standards, transparency and reporting to the Assembly.

20. **Phased implementation roadmap.** A phased roadmap is proposed: an establishment phase (first three to six months) covering adoption of the terms of reference by the inaugural Assembly, nomination of representatives and focal points, confirmation of the Secretariat and establishment of the initial Working Groups; a foundations phase (months six to twelve) covering a regional landscape analysis, the first workplan, the regional Transformation Toolbox and the first learning visits and project collaboration agreements; and a delivery phase (years one to two) covering Working Group deliverables, potentially fellowships and exchanges, publication of outputs and the first annual report to the Assembly.

## The way forward

21. Based on the demonstrated Member State demand, the challenges identified and the consultative process undertaken, the following strategic priorities are proposed to establish and operationalize SEA-DAC:

22. **Secure a Member State mandate.** Endorsement by the Regional Committee would provide SEA-DAC with the legitimacy, continuity and accountability required for sustained regional collaboration and would authorize the convening of the inaugural Assembly of Members to adopt the terms of reference, establish working groups and approve the workplan.

23. **Operationalize the governance architecture.** Member States would designate national Agencies with national oversight of digital health and AI, nominate senior representatives to the Assembly and Steering Group, and appoint national focal points.

24. **Deliver early, visible results.** Early activities could include, subject to priorities agreed by the inaugural Assembly, a regional landscape analysis of digital health and AI actors, initiatives and tools; the standing up of a regional repository of digital public goods and standards profiles; structured learning and immersion visits responding to already-articulated Member State requests; and the first project collaboration agreements matching standing offers of support to priority country needs.

25. **Ensure sustainable financing.** The Secretariat and network activities would be supported through a transparent and sustainable funding model drawing on contributions from Member States, GIDH, development partners, international financial institutions and other non-commercial sources, with clear safeguards to manage conflicts of interest, ensure operational independence and maintain alignment with regional priorities and national sovereignty.

26. **Maintain complementarity and alignment.** SEA-DAC would report into GIDH global processes and maintain active alignment — including reciprocal participation and shared knowledge products — with existing global and regional mechanisms, ensuring that regional activity reinforces rather than duplicates the wider digital health ecosystem.

27. **Monitor, evaluate and learn.** The network would adopt a light-touch monitoring, evaluation and learning approach: an annual work plan with measurable action points; periodic reporting from Working Groups to the Steering Group and Assembly; assessment of the impact of implemented actions; and publication, where possible, of summaries of outputs to disseminate regional best practice.

## Conclusions

28. The digital transformation of health systems presents the Region with a time-limited window of opportunity. Member States have made considerable progress and have clearly articulated demand — through formal requests, standing offers of support and regional consultations — for a structured, Member State-led mechanism to accelerate collaboration on digital health, data and AI under the Global Initiative on Digital Health.

29. SEA-DAC responds directly to this demand. By adopting validated governance and joint-learning practices from established mechanisms, by placing national health agencies at the centre of decision-making, and by separating Member State-led governance from delivery through WHO's established partnership instruments, the network offers a credible, non-duplicative and sustainable platform for equitable access to trusted, safe and interoperable digital health and AI solutions across the Region.

30. Member States, WHO and partners are invited to act on the momentum generated by the consultative process to date. The establishment of SEA-DAC, with a clear mandate from the Regional Committee, would convert demonstrated demand and standing offers of support into timely, equitable and country-led implementation, in support of universal health coverage and the health-related Sustainable Development Goals.

## Lead questions on key asks for the Regional Committee

31. The Regional Committee may wish to consider the following questions en route to reaching its recommendations and decision:

- (1) Should the Regional Committee endorse the establishment of the South-East Asia Digital Health and Artificial Intelligence Collaborative Network (SEA-DAC) as the dedicated regional implementation arm of the Global Initiative on Digital Health for the South-East Asia Region?
- (2) Should the Regional Committee support the proposed governance architecture and terms of reference of the network, to be adopted, with any amendments, by the inaugural Assembly of Members?
- (3) Are Member States prepared to designate national agencies with national oversight of digital health and AI, to nominate senior representatives and national focal points, and to participate actively in the Working Groups within the proposed timelines and contribute to identification of regional priorities and the network's first biennial workplan?
- (4) Should the Regional Committee agree that WHO, through the Regional Office for South-East Asia, serve as the Secretariat of the network and operationalize technical delivery through project collaboration agreements and related instruments?
- (5) What guidance should the Regional Committee provide on sustainable financing for the network and on ensuring continued complementarity with existing global and regional mechanisms?